Ibandronic acid

Ibandronic acid is used to treat and prevent problems caused by the effect of cancer in the bones.

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What is ibandronic acid?

Ibandronic acid belongs to a group of drugs called bisphosphonates. It can be used to:

- prevent bone weakness or bone fractures caused by breast cancer that has spread to the bones
- treat high levels of calcium in the blood (hypercalcaemia) that can be caused by cancer.

It is best to read this information with our general information about bisphosphonates and the type of cancer you have.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the electronic Medicines Compendium (eMC) website, which has patient information leaflets (PIL) for individual drugs.

The effect of cancer on the bones

Bone cells constantly renew themselves. This helps bones keep their strength and shape.

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There are 2 types of cell inside the bones. These are called:

- osteoclasts, which break down and remove old bone
- osteoblasts, which build new bone.

As we get older, osteoclasts begin to break down more bone than osteoblasts make to replace it. This means our bones slowly become thinner (less dense). In some people, too much of the bone density is lost and this increases the risk of osteoporosis and bone fractures.

Some secondary bone cancers make a substance that causes osteoclasts to destroy more bone than can be replaced. The affected bone becomes weak and painful and can break more easily.

Bones contain calcium, which gives them strength. A bone affected by cancer may cause calcium from the bones to be released into the blood. A raised level of calcium in the blood is called hypercalcaemia. This may cause symptoms including:

- feeling sick (nausea)
- being sick (vomiting)
- feeling thirsty
- needing to pee (pass urine) more often
- constipation
- tiredness
- irritability
- confusion.

How ibandronic acid works

Ibandronic acid reduces the activity of the cells that break down the bone (osteoclasts). This can help reduce pain and strengthen the bone.

Ibandronic acid also reduces the amount of calcium that is lost from the bones. This helps calcium levels in the blood return to normal.

How ibandronic acid is given

When you have your treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

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During the course of your treatment, you will have regular blood tests. This is to check that it is safe for you to have treatment.

You will meet with a doctor, nurse or pharmacist before you have treatment. They will talk to you about your blood results and ask how you have been feeling.

You may have ibandronic acid by a drip (intravenous infusion) into a vein, or as a tablet. Ibandronic acid can be given with other cancer treatments.

If you are having this treatment to prevent bone problems, you usually need to take ibandronic acid for at least 6 months for it to have the full effect on your bones. Your doctor or nurse can tell you more about this. After that, you can usually take it for as long as it is working well.

If you are given it as an intravenous infusion, you usually have in the outpatient department at the hospital, every 3 to 4 weeks. The infusion can take up to 1 hour. If you are having intravenous ibandronic acid into the vein to treat high calcium levels, you usually only have one dose.

Having ibandronic acid by drip (intravenous infusion)

You may have the ibandronic acid infusion through 1 of the following:

- a cannula – a short, thin tube the nurse puts into a vein in your arm or hand
- a central line – a fine tube that goes under the skin of your chest and into a vein close by
- a PICC line – a fine tube that is put into a vein in your arm and goes up into a vein in your chest
- an implantable port (portacath) – a disc that is put under the skin on your chest or arm and goes into a vein in your chest.

Taking ibandronic acid tablets

If you are having ibandronic acid as tablets, your cancer doctor, nurse or pharmacist will give you the tablets to take at home. Always take ibandronic acid tablets exactly the way you are told to. This is to make sure they work as well as possible for you.

Ibandronic acid can attach itself to certain substances in food, drinks and medicines. If this happens, it may not be absorbed properly and may not work as well. This means you need to do the following:

- Take the tablet first thing in the morning.
- Take it when you have an empty stomach this means at least 6 hours after you last had anything to eat or drink except water, or had any other medicines or supplements.
- After you take the tablet, do not eat, drink anything (other than tap water), or take any other medicines by mouth for at least 30 minutes to 1 hour. The exact length of time you need to wait will depend on your dose of ibandronic acid. Your doctor or pharmacist will give you more information about this.
- Do not suck, chew or crush the tablet.
- Swallow the tablet whole, with a full glass of plain tap water. If the water in your area is known to have hard water (possible high levels of calcium in the water), it is advised that you use bottled water with low mineral content. If you cannot swallow or have difficulty swallowing, you must tell your doctor or pharmacist.
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You need to sit up straight or stand up when you swallow the tablets. This is to make sure the tablet is washed down well. This stops it from irritating your gullet (oesophagus). The gullet is the tube that goes from your mouth to your stomach.

- Stay sitting up straight or standing for 1 hour after you take the tablet. If you are in bed, sit upright supported with pillows.
- If you miss a dose, contact your cancer doctor, nurse or pharmacist for advice on when you should take it.

Ask your cancer doctor, nurse or pharmacist if you are not sure how to take the tablets.

Lactose

Ibandronic acid tablets contain lactose. If you have a lactose allergy or intolerance, talk to your doctor, nurse or pharmacist before taking this treatment. They can give you more information.

Dental check-up before you have ibandronic acid

Rarely, this treatment can affect the jawbone. You can read more about this in the side effects section below.

Keeping to a regular oral hygiene routine keeps your mouth and teeth healthy. This reduces your risk of jaw problems. You usually need a dental check-up to check for any problems before you start treatment.

If you need urgent treatment to lower high calcium levels in your blood, you do not usually need to have a dental check-up first.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Other cancer treatments may cause different side effects. If you are also having other cancer treatment, you may have other side effects.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

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Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Side effects while treatment is being given

Allergic reaction

Rarely, some people have an allergic reaction while having this treatment when it is given into the vein (intravenously – IV). Signs of a reaction can include:

- feeling hot or flushed
- shivering
- itching
- a skin rash
- feeling dizzy or sick
- a headache
- feeling breathless or wheezy
- swelling of your face or mouth
- pain in your back, tummy or chest.

Your nurse will check you for signs of a reaction during your treatment. If you feel unwell or have any of these signs, tell them straight away. If you do have a reaction, they can treat it quickly.

Sometimes a reaction happens a few hours after treatment. If you develop any of these signs or feel unwell after you get home, contact the hospital straight away on the 24-hour number.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

Possible side effects of ibandronic acid

Flu-like symptoms

Some people have flu-like symptoms, such as:

- a high temperature
- chills

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- fatigue (tiredness)
- pains in your muscles or joints.

Tell your doctor if these symptoms do not get better within 2 days or are getting worse. It may help to take mild painkillers, such as paracetamol. Contact the hospital on the number you have been given. Your doctor, nurse or pharmacist can give you advice.

Increased bone pain

Sometimes pain in the affected bone gets worse for a short time when you start taking this treatment. If this happens, your doctor can prescribe painkillers to help.

Increased risk of infection

This treatment can increase the risk of getting an infection. Symptoms of an infection can include:

- feeling cold and shivery
- having a higher or lower than normal temperature
- aching muscles
- headaches
- feeling generally unwell.

Depending on where the infection is you might have other symptoms.

Contact the hospital straight away on the number you have been given if:

- your temperature goes over 37.5˚C (99.5˚F)
- your temperature goes below 36°C (96.8°F)
- you suddenly feel unwell, even with a normal temperature
- you have symptoms of an infection.

Feeling sick (nausea)

You may feel sick or be sick (vomit) while having this treatment. This is usually mild. Your doctor can prescribe anti-sickness drugs to help prevent or control sickness.

Take the drugs exactly as your doctor, nurse or pharmacist tells you to. It is easier to prevent sickness than treat it after it has started.

Tummy pain

If you have pain in your tummy (abdomen) that is severe or does not go away, contact the hospital straight away on the 24-hour number you have been given.

Diarrhoea

This treatment may cause diarrhoea. Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

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If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Headaches and dizziness

You may have headaches or feel dizzy at times when taking this treatment. Tell your doctor if you notice these symptoms.

Muscle or joint pain

You may get pain in your muscles or joints for a few days after treatment. If this happens, tell your doctor, nurse or pharmacist. They can give you painkillers and advice. They can also tell you if any of the painkillers you usually take are suitable.

Tell them if the pain does not get better. Having warm baths and resting regularly may help.

Sore throat or indigestion

If you take the ibandronic tablets, they may irritate the throat or gullet (oesophagus). This is the tube that goes from the mouth to the stomach. If you have pain or problems with swallowing or indigestion that is new or getting worse, contact the hospital as soon as possible for advice. Tell your doctor before taking any more ibandronic acid tablets.

Feeling tired (fatigue)

Feeling tired is a common side effect. Try to pace yourself and plan your day so you have time to rest between activities.

Being physically active can help to manage tiredness and give you more energy. It also:

- helps you sleep better
- reduces stress
- improves your bone health.

If you feel sleepy, do not drive or operate machinery.

Numbness or tingling

You may notice numbness or tingling around the mouth, or in the fingers and toes. Contact your doctor or the hospital straight away if you have these symptoms. This may be caused by low levels of calcium in your blood. You will have regular blood tests to check your calcium levels.

Your doctor may ask you to take calcium and vitamin D supplements, unless you are having this treatment to lower the levels of calcium in your blood. Your doctor will let you know if you need any supplements.

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Changes to your taste

Some foods may taste different or have no taste. Try different foods to find out what tastes best to you. You may also get a bitter or metallic taste in your mouth. Your doctor, nurse or pharmacist can give you advice. It might help to try:

- sucking sugar-free sour or boiled sweets
- eating cold foods
- eating sharp-tasting fresh fruit.

Taste changes usually get better after treatment ends. We have more information about coping with changes to taste.

Skin rash

This treatment can cause a rash. Always tell your doctor, nurse or pharmacist about any skin changes. They can give you advice or prescribe creams or medicines to help.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Effects on the kidneys

This treatment can affect how the kidneys work. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your kidneys are working. Contact the hospital on the 24-hour number if you:

- have blood in your urine (pee)
- are passing less urine or peeing less often than usual.

Drinking fluids helps protect your kidneys. The advice is usually to try to drink at least 2 litres (3½ pints) of fluid each day. But follow any advice from your doctor, nurse or pharmacist about how much is right for you.

Effects on the eyes

Rarely, this treatment can cause eye problems. If you have any of these symptoms during treatment or after it finishes, contact the hospital on the 24-hour number straight away. Contact them if:

- you notice any change in your vision, such as double vision or things appear blurry or faded
- your eyes get red and inflamed (conjunctivitis)

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- you have pain in your eyes
- your eyes are sensitive to light

You may need to have an eye check with a specialist eye doctor (ophthalmologist).

Pain in the thigh, hip or groin

Rarely, people having this treatment develop a break (fracture) in their thigh bone without an obvious cause. Sometimes both thigh bones are affected.

If you have any thigh, hip or groin pain, tell your doctor. Let them know that you are taking bisphosphonates. They can arrange tests to check the thigh bones for any signs of bone weakness or fracture.

Jaw problems

Rarely, this treatment can affect the jawbone. Healthy bone in the jaw becomes damaged and dies. This is called osteonecrosis of the jaw (ONJ). It can cause:

- pain
- loosening of the teeth
- problems with the way the gums heal.

The risk of jaw problems is higher after some types of dental treatment. It is also higher in people who smoke, have gum disease or dentures that do not fit well.

It is important to avoid having dental treatment that could affect your jawbone when you are having bisphosphonates. This includes having a tooth or root removed or dental implants put in. You can still have fillings, gum treatments or a scale and polish.

To reduce your risk of developing jaw problems, your doctor will advise you to:

- have a full dental check-up before starting treatment
- look after your teeth and gums during treatment (ask your dentist for advice)
- tell your dentist you are taking a bisphosphonate before having any dental treatment.

Tell your cancer specialist and dentist straight away if you at any time you develop:

- pain
- swelling or redness in your gums
- numbness or heaviness in your jaw
- loose teeth.

Ear problems

Very rarely, bones in the outer ear may be affected by this treatment. Always tell your doctor if you have any:

- ear pain
- discharge from your ear

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- ear infections.

You should also let them know if you notice any other changes in your ears or hearing.

Serious skin changes

Very rarely, this treatment can cause a serious skin reaction that needs to be treated immediately in hospital.

Do not take any more of this treatment and contact the hospital straight away on the 24-hour number if you have any of these symptoms:

- a skin rash that is spreading
- blistering or peeling skin
- flu-like symptoms, such as a high temperature and joint pain
- sores on your lips or in your mouth.

Other information

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- herbal drugs and complementary or homeopathic therapies
- recreational drugs – for example, cannabis.

Going into hospital

If you are admitted to hospital for a reason not related to the cancer, it is important to tell the doctors and nurses that you are having treatment with bisphosphonates. You should tell them the name of your cancer specialist. This is so they can talk to them for advice.

Contraception

Your doctor, nurse or pharmacist will advise you not to get pregnant or make someone pregnant while having this treatment and for some time afterwards. If you are pregnant, think you might be pregnant or are planning to have a baby, you must tell your doctor as soon as possible. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

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Breastfeeding

You are advised not to breastfeed while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References

Visit the electronic Medicines Compendium (eMC) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

Cancer Information Development team

Author

Date reviewed

Reviewed: 01 January 2025 | Next review: 01 January 2027

Our cancer information meets the PIF TICK quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about how we produce our information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly

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- make sure important points are clear.

We use gender-inclusive language and talk to our readers as ‘you’ so that everyone feels included. Where clinically necessary we use the terms ‘men’ and ‘women’ or ‘male’ and ‘female’. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information here.

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.

Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.

Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don’t want to call.

Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.

Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help

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